PHL204: Philosophy in Everyday Life

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Outline

The Question

Ethics of MAID

Objections to MAID

Topic

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Today's question

Is MAID ethical?

Eligibility

To be eligible, you must meet all the following criteria:

- Be at least 18 years old & eligible for healthcare in Canada
- Be mentally competent (be able to make your own health care decisions)
- Have a "grievous and irremediable medical condition"
- Make a voluntary request for MAID
- Give informed consent to receive MAID

Medical Conditions

To count as having a "grievous and irremediable medical condition" you must meet all the following criteria:

- have a serious illness, disease or disability
- be in an advanced state of decline that cannot be reversed
- experience unbearable physical or mental suffering that cannot be relieved under conditions you deem acceptable

- it is not required that you have a fatal or terminal condition
- if your only condition is a mental illness this doesn't qualify until 2027

(Information from the Canada government website on MAID)

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Moral Case for Patients Rights

The moral case for patients to have the right to seek medically assisted death rests on two principles:

- we have to respect the autonomous choices of all persons as long as those choices do not harm others (this requires competence in being able to decide for yourself)
- we want to promote the well-being of all persons and those that are suffering (whether or not it is terminal) may have their well-being served better by death

If so, then there is a moral case to allow patients to seek medically assisted death

Autonomy

Autonomy: the capacity to make your own decisions free of coercion or dependence

Autonomy

Generally, we want to take control of the important decisions in our lives

We want to make these decisions according to our own beliefs about how to live

If you can decide how and when you die, this might be something you want to take autonomous control over

Autonomy

It is understandable to be concerned about how our life will look at the very end

We fear that death may involve a long period of suffering or the loss of dignity

So, we should have the right to choose our moment of death before we reach this stage

And if we want patient-centered care, don't we want patients to able to decide this?

Well-being

The other condition is an appeal to well-being

Some conditions of life involve deep suffering (what the Canadian guidelines says is the condition of "unbearable physical or mental suffering that cannot be relieved under conditions deemed acceptable")

Well-being

If somebody is in this condition, then it may well follow that their well-being is promoted if they are allowed to seek MAID

Autonomy and Well-being

These two conditions (autonomy and well-being) must be met in order for a person to qualify for the right to medically assisted voluntary death

This means: the two conditions are individually necessary and jointly sufficient

Necessity and Sufficiency

Sufficiency and Necessity are logical relations

For example,
X = Being in Toronto
Y = Being in Canada

- X is sufficient for Y if, once you have X, you get Y
  - So, being in Toronto suffices to be in Canada
- Y is necessary for X if you cannot have X without Y
  - So, being in Canada is necessary to be in Toronto

The two are inter-defined: if some X is sufficient for some Y, then that same Y is necessary for that X

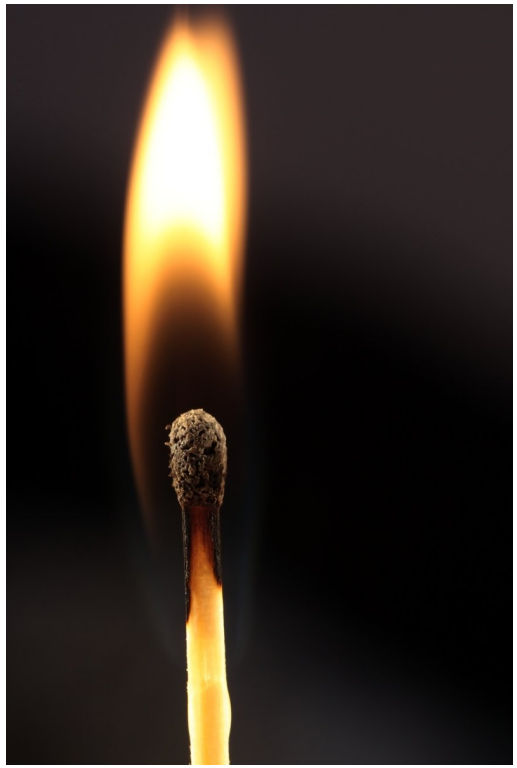
Necessity and Sufficiency

Generally, for any particular state to obtain, it requires a number of conditions to be met

If you want to light a match, you need:

- A: Some combustible fuel
- B: Oxygen
- C: Some spark or heat source

In this case: ABC are individually necessary and jointly sufficient to light a match



Necessity and Sufficiency

Another example, if you want to play a game of soccer, you need:

- A: Soccer ball
- B: Goal posts
- C: Enough players



These conditions again, must each be met so they are individually necessary but only if they are all met do you get a game of soccer, so they are jointly sufficient

Autonomy and Well-being

Back to the morality of MAID

In order for the state patient has the right to seek MAID to obtain, the following two conditions need to be met:

- A: the patient makes an autonomous decision to receive MAID
- B: receiving MAID will aid the well-being of the patient

So, neither of these conditions is enough by itself for somebody to have this right (but why not?)

Autonomy alone

Why is it not enough for autonomy considerations alone to give one the right to receive MAID?

If this is enough to have the right to MAID, then somebody could request MAID in a situation in which, for example, they are "tired of life" and we would have no means of resisting their request

After all, if the autonomous decision alone is all that matters, then this patient has reached the threshold

So this condition cannot suffice for the right, it can only be necessary for the right

Well-being alone

Why is it not enough for well-being considerations alone to give one the right to receive MAID?

If this is enough to have the right to MAID, then somebody could receive MAID in a situation in which they cannot make any decisions at all, or have no competence to make decisions

After all, if the well-being of the patient is all that matters, then this patient has reached the threshold

So this condition cannot suffice for the right, it can only be necessary for the right

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Palliative Care

The first challenge is the following:

It is not necessary to provide a right to seek MAID because with the right palliative care we can alleviate suffering

Palliative Care

Palliative Care = healthcare approach that optimizes well-being and aims to reduce or even eliminate suffering

Question: is the availability of this kind of care enough of a reason to deny patients the right to seek MAID?

Palliative Care

Some issues with this challenge:

- It is not always obvious what kind of care a particular patient requires, so there is a period of trial and error to get to the right solution, which may involve prolonged periods of suffering
- In general, even the best of care cannot relieve all suffering and pain
- This is not a panacea

Should we be demanding that patients go through this?

Palliative Care

Some issues with this challenge:

- High quality palliative care often involves strong side-effects like incontinence or nausea
- It usually also requires patients to spend long periods of the day semi-conscious in states of drowsiness

Should we be demanding that patients go through this?

Palliative Care

Some issues with this challenge:

- Of those that request MAID in Canada, nearly 80% were in palliative care at the time
- So, it seems unlikely that this is a permanent solution for those suffering serious illness

Palliative Care

Some issues with this challenge:

- Not everyone wants palliative care
- Some want to die in the dignity of their home and not have to spend their days in a foreign bed in a hospice receiving constant doses of morphine to keep the pain away

Should we be demanding that patients go through this?

Palliative Care

Some issues with this challenge:

- Many who seek MAID suffer not just from pain but also suffer in their forced dependence on others and/or on life-supporting machinery
- So even if palliative care can reduce the pain, this will not reduce the suffering for these patients

Other Treatments

The second challenge is this:

Many patients are not required to try out all other treatments before they exercise their right to seek MAID

But this might mean that there might be other treatments that may have helped them, and they may not have exercised this right in that case

Other Treatments

However, patients always retain the right to refuse treatments

If we were to force patients into trying other treatments without their consent, this would amount to a form of paternalism

Other Treatments

Paternalism = a system in which healthcare providers (primarily doctors) make decisions on patient treatment based on their own judgment rather than on the wishes or preferences of the patient

In general, the healthcare system in Canada is anti-paternalist

So, we cannot require patients to try other treatments first, their right to MAID is not dependent on this

Other Treatments

Moreover, given that we already and uncontroversially allow patients to refuse life-prolonging treatment, leading to eventual death, then why not also let them have the right to seek MAID too?

Letting Die v Ending a life

This leads to the third challenge: the difference between passive and active euthanasia

It is generally considered morally permissible to let patients die if they refuse life-prolonging treatment, because no steps are taken to help them (passive euthanasia)

However, one might argue that it is immoral to take active steps—to form an intention and act to bring it about so that a patient's life is ended (active euthanasia)

Letting Die v Ending a life

The problem with this distinction is that it is not always clear how to draw a line between these two situations

When exactly are we acting towards an end and when are we just allowing something to happen? Isn't it all just actions that we perform?

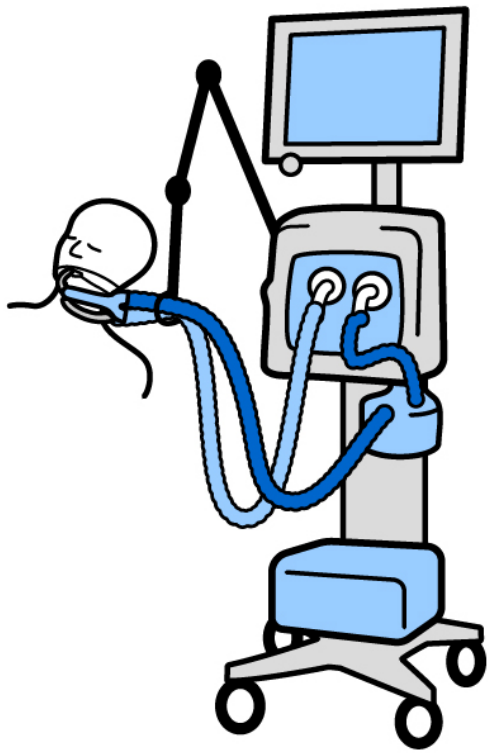
Letting Die v Ending a life

Scenario 1

A dying patient is being kept alive by a respirator machine

After discussion with the family, the doctors pull the plug on the machine

The patient dies shortly thereafter



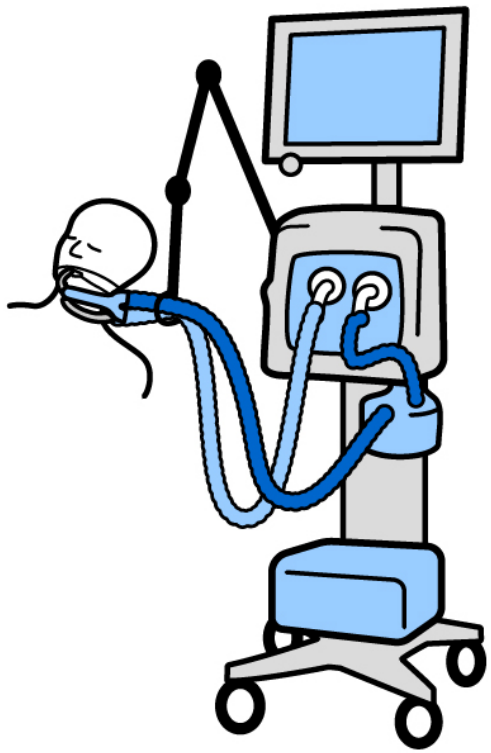
Letting Die v Ending a life

Scenario 2

A dying patient is being kept alive by a respirator machine

After discussion with the family, the doctors let the last oxygen tank run out

The patient dies shortly thereafter



Letting Die v Ending a life

Is there a big moral difference between these two scenarios?

If not, then there is not much of a distinction between letting a patient die and ending their life *especially if the patient has competently requested it*

Letting Die v Ending a life

In general, moral theories that consider only the outcome of an action are called consequentialist

- on this view, all that matters is the consequence of the action, and given that we have the same consequence in either case, there is no real moral difference between the two scenarios

Letting Die v Ending a life

Indeed, sometimes it can be more cruel to let somebody suffer for a prolonged period until they die rather than relieving their suffering by helping end their life sooner, if they ask for it

If the intention is the same either way, then the doctor should go for the option that will relieve suffering sooner—this will have the better consequences

Letting Die v Ending a Life

In general, moral theories that consider how an outcome is produced, rather than its consequences, are called deontological

- on this view, what matters is following the right moral rules, among which is the moral duty to not kill, so this establishes a difference between the two scenarios

Letting Die v Ending a Life

One way of pushing this point: it doesn't matter if a patient consents to MAID, because nobody can treat a person instrumentally, not even the person themselves

So, autonomously choosing MAID does not eliminate the distinction between letting die and ending a life because autonomy is not the highest moral principle

Rather, treating persons as ends-in-themselves is the highest moral principle

Means to an ends and Ends in themselves

To treat somebody as a means to an end is to treat them as an instrument towards an outcome

So, treating a person's life as a disposable means towards the end of avoiding suffering is morally impermissible, even if the patient wants it!

To treat somebody as an end in themselves is to recognize that they have an inviolable moral dignity and worth that cannot be instrumentally traded away

We have to treat them as a person and not a thing

And that means that we cannot end their life, even if they volunteer for it

Letting Die v Ending a Life

Question: what do you think? Is there a deep moral difference between letting a patient die and actively ending their life when they volunteer for it?

Competence and Voluntariness

This also leads to the fourth challenge:

How can we ever have enough evidence that a person's decision to seek MAID is competent, enduring, and genuinely voluntary?

Competence and Voluntariness

There are a few separate issues here:

- The issue of competence
- The issue of coercion

Competence

It is certainly true that there may be patients who are so affected by pain that they cannot think straight

These patients may even be mentally confused and not able to think rationally about their situation

Most supporters of the right to MAID would in these cases agree that this patient does not have a right

However, most patients who seek MAID are not in this position

Coercion

The worry about coercion can take a number of different forms:

- We may worry about whether patients are being coerced into MAID because doctors are able to initiate the conversation on the possibility of seeking MAID
- We may worry about whether patients are being coerced into MAID because of social determinants: the waiting list for housing, treatment etc is intolerably longer than the wait time to request and receive MAID

Initiating the conversation

The worry here is the following:

If the doctor initiates the conversation on MAID, then that might mean that a patient is no longer voluntarily seeking out MAID

- they may, for instance, feel pressure to agree with a person of authority like a doctor

Initiating the conversation

However, we ought to be consistent with this worry

Is it a problem if a doctor is telling a patient about a cancer diagnosis and initiates the conversation about all the various treatments that are available?

What is different about these cases?

Social Determinants

One of the major worries is that patients may be opting for MAID because they are waiting for housing or pain treatment or disability benefits, and these waitlists are intolerably long

Dr John Maher: "killing people while they are on waitlists is profoundly immoral"

Social Determinants

It is true that MAID waitlists are a lot shorter than waitlists for pain treatment clinics, long term care homes, etc

Question: Does this raise a moral problem about MAID?

Social Determinants

Why is this a problem?

If somebody requests MAID because they are waiting too long to get the right treatment, then they may well not have made this choice if the resources were available to them sooner

Is it right to put this patient through MAID because society has failed them?

Moreover, has the lack of resources coerced them into choosing MAID? Was this really a free and voluntary choice?

Social Determinants

Two possible replies to this problem:

- Concede the problem: yes, society should have better resources available, but that doesn't rule out all cases of MAID in principle, some people still want MAID even with all the best treatment already available to them

- Anti-paternalism: why should an individual have to suffer because of society's failings? We should respect the choices of individuals even if society is imperfect

Social Determinants

Notice that even if it is true that some patients that seek MAID are doing so for non-medical reasons, this doesn't affect the argument for the right to MAID:

It says that you have to voluntarily seek it and it must be for your medical well-being in order to have the right to it—so those that are doing this, don't have the right anyway

In principle, the right still stands

However, if enough people start seeking MAID for non-medical reasons, then does that change our principles? Where should we draw the line?

Mental Health

The final challenge for MAID is the following:

Is it appropriate for MAID to be available for those suffering from mental health issues alone?

After all, it is extremely difficult to know which mental health conditions can be reversed and which cannot be

How can these conditions count as a "grievous and irremediable medical condition"?

Mental Health

One reply to this problem: it might be that there are some conditions that are not treatable, but surely patients with these conditions ought to have the same rights as everyone else?

However, it is difficult to settle a question on what is treatable or not: mental health conditions are not in a strict binary

It might be that we can we can make their situation more manageable regardless—by bringing them love, support and care as a society

Question: what are the social duties that we have to each other? Does this change how we think about the ethics of MAID for mental health patients?